TRAUMA-07 | For therapists | Two separate age-based pathways

Relationships, beliefs and appropriate sharing

1 | Purpose, fit and boundaries

Purpose of this unit

Explore relationships and sharing suited to the person's needs. Children and teenagers focus on communication with a suitable supporter; adults use a discussion map of CPT's five belief domains.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

Sharing a story, becoming close, forgiving or trusting someone is not required. Five topics do not fully cover five CPT components. Current harm is not an interpretation to replace with positive thinking.

Reusing existing material

FAMILY and SOC offer language for relationships and requests. They do not replace adapted joint TF-CBT sessions or full CPT belief-domain work.

A fictional example for discussion

Fictional example: Ruth wants support from her sister while keeping details private. With the therapist, she explores necessary information and valued boundaries; full disclosure is not required for help.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | Age-based pathways

CH / TE | Support within a TF-CBT pathway

CH/TE sheets prepare supporter conversations and clarify needs and boundaries. Joint-session planning and supporter preparation follow TF-CBT; this sheet does not require narrative sharing.

AD / OA | Support within a CPT pathway

AD/OA sheets offer an opening question for each CPT belief domain. Full mapping, approach and depth still require a manual and reviewer; these are not five complete treatment modules.

Parallel work with a suitable support person

Assess suitability and safety before sharing. Support listening, nonblaming responses and boundaries. Do not include an unsuitable person merely because they are a parent, partner or relative.

Using W and P sheets

Choose the age version and relevant questions; completing every sheet is not a condition for progress. W accompanies professional conversation. P records only pre-agreed follow-up. Unit number does not mean session number.

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

Possible difficulties

Disagreement with a supporter does not prove youth noncooperation. Assess safety, power and privacy. For adults, do not assume a wish for distance is a stuck point.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children, approximately ages 6 to 11

CH joint conversations involve only suitable, safe supporters. Prepare each side separately as needed; do not ask children to manage supporters' emotions or justify their experience.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

TE respects content choices and privacy within safeguarding duties. A joint session does not automatically authorize sharing every treatment detail.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

AD's five W questions map safety, trust, power and control, esteem and intimacy. Choose a domain for full professional work; five completed lines do not complete CPT.

OA | Older adults

OA respects roles, partnerships, loss, interdependence and autonomy without age assumptions. Closeness need not involve partnership or touch; the person defines valued relationships and boundaries.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

P records support and learning agreed in treatment. It sets no exposure, trauma narrative, dosage or component sequence. Without a plan, bring questions to the session. CH/TE sheets do not replace separate supporter work; AD/OA sheets are not official CPT materials.

At the next session

Check request and boundary clarity and support changes needed. In CPT, return to chosen work domains, not merely whether a conversation occurred. All domains need not be completed in one session.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

Treatment-manual mapping status

Proposed: TF-CBT for children/teens; CPT for adults/older adults. Full manuals remain unmapped. Age, rights, supporter materials and separate-guide needs await review. Original support drafts, not an approved protocol.

www.nctsn.org/interventions/trauma-focused-cognitive-behavioral-therapy
tfcbt.org/about/
www.ptsd.va.gov/professional/treat/txessentials/cpt_for_ptsd_pro.asp

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